



**CHRISTIAN MEDICAL COLLEGE
VELLORE - 4
DEPARTMENT OF ENT UNIT 3
RHINOLOGY AND ANTERIOR SKULL BASE UNIT**

**DISCHARGE SUMMARY
PRELIMINARY REPORT**

Consultants

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Dr. Lisa Mary Cherian MS., DLO., PhD.,

Associate Professor

Name : MINU RANI SHARMA MRS

Hospital Number : 223074P

Age : 47

Sex : Female

Ward :

Admitted On : 15-Sep-2021

MRDNo : 692324J

Address : QT.NO TYPE II-22/210 UCIL COLONY JADUGODA
JAMSHEDPUR

EAST SINGHBHUM

JHARKHAND

Pincode : 832102

Diagnosis:

BILATERAL CSF LEAK

HYPOTHYROIDISM

HYPERTENSION

DIABETES MELLITUS

History

Minu Rani, 47 year old from Jharkhand presented with 9 month history of left sided watery nasal discharge. She was unable to sniff back the discharge and also had salty sensation in her throat. One month ago she also noticed intermittent watery nasal discharge from her right nasal cavity.

There is no history of trauma to nose or surgery in the past

No history of bleeding, sneezing, facial pain, headache, anosmia.

No ear and throat complaints

She is a known case of hypothyroidism, Hypertension and Diabetes mellitus on medications.

On Examination

Ear-

Bilateral pre auricular, post auricular region normal

EAC normal bilaterally

Bilateral tympanic membrane intact- dull

Rinnies -bilateral positive

Weber not appreciated

Nose-

External framework normal

Bilateral vestibulen otmal

Septum- DNS to left

Bilateral inferior turbinate hypertrophy

Left sided active nasal watery discharge seen

No active watery discharge on right side

No para nasal sinus tenderness

Oral- Bilateral grade 1 tonsils

Posterior pharyngeal wall normal

Neck- normal
Eyes- Vision, eye movements normal

Investigations

27/08/2021	HAEMOGLOBIN	9.7	GM%	
27/08/2021	PT WITH INR	13.8	SECS	
	PATIENT	11.7-16.1	SECS	
	NORMAL RANGE	0.99		
	INR		SECS	
27/08/2021	1/2Pt+1/2 Cont			-
	APTT	31.5	SECS	
	PATIENT	27.8-40.4		
	NORMAL RANGE		SECS	
	1/2Pt+1/2 Cont			
15/09/2021	BLOOD GROUPING & RH TYPING	O RH POSITIVE		
	ANTIBODY SCREENING NEGATIVE	45330.87.2		
27/08/2021	CREATININE	0.64	mg%	0.5-1.4
27/08/2021	GLUCOSE FASTING	124	mg/dL	70-99
27/08/2021	GLUCOSE 2HRS POST FOOD	210	mg/dL	<140
27/08/2021	CSF GLUCOSE	122	mg/dL	40-70
27/08/2021	CSF PROTEIN	48	mg/dL	15-45
27/08/2021	CSF CHLORIDE	135 R	mEq/L	118-132
27/08/2021	T4 (TOTAL T4 AND FREE T4)			
	T4	10.4	ug%	Adults 4.5
- 10.9				
	FTC	1.25	ng%	euthyroid
0.89 - 1.76				
27/08/2021	TSH	2.064	uIU/ml	0.3-4.5
27/08/2021	LIPID PROFILE - FASTING			
	CHOLESTROL - TOTAL	160	mg%	< 200
	TRIGLYCERIDE SERUM	144	mg%	<150
	CHOLESTROL - HDL	50	mg%	40-60
	CHOLESTROL - LDL	88	mg%	<100
27/08/2021	HBA1 C (GLYCOSYLATED Hb)	6.2	%	<5.7
30/08/2021	ECG (CARDIOLOGY)			
13/09/2021	BLOOD BORNE VIRUS SCREEN			
	HIV	NEGATIVE		
	HbsAg	NEGATIVE		
	HCV	NEGATIVE		
14/09/2021	QUALITATIVE PCR FOR SARS COV-2 [ROUTINE]	NEGATIVE		
27/08/2021	OPTIC NERVE SHEATH DIAMETER			
27/08/2021	RIGID NASAL ENDOSCOPY: ? LEFT CSF LEAK ,1) medial to middle turbinate 2) anterior ethmoids.			
	Study Date: 02-Sep-2021 - Report Approved			

MRI AND CT CSF RHINORRHOEA

FINDINGS:

Tiny defect is seen in the right cribriform plate measuring 1.2 mm with continuation of CSF signal into the anterior ethmoid air cells. (Im: 102.49)

Tiny defect is seen in the left cribriform plate measuring 1.5 mm with continuation of CSF signal into the right ethmoid air cells (Im: 102.53).

CSF signal intensity is seen in the left hemisphenoid. No obvious defect seen - could represent pooling of CSF.

The brain parenchyma is of normal signal intensity and morphology.
The basal ganglia, thalami, brainstem and cerebellar hemispheres are normal.
The ventricular system is within normal limits.
Note made of empty sella.
Bilateral optic nerves are tortuous with peri-optic CSF flare.
Bilateral maeckel's caves prominent

IMPRESSION: A 47-year-old lady with CSF rhinorrhoea (left > right), MRI and CT of the anterior skull base shows:

"Bony defects in bilateral cribriform plates as described with extension of CSF signal into anterior ethmoid air cells.

"CSF signal intensity in the left hemisphenoid sinus. However no obvious defect seen - could represent pooling of CSF.

"Features of raised intracranial tension.

Dr. Phaneendra Rao

Operation Done

BILATERAL ENDOSCOPIC CSF LEAK REPAIR WITH TP SHUNT DONE UDNER GA ON 16/9/2021

Operative Findings

Two defects noted on the left side, both at the region of cribriform plate
One defect was 1 cm posterior to the axillar of middle turbinate
Another defect was medial to the superior turbinate
Meningocele seen protruding from the both the region of defect.
Active csf leak noted.
Left anteriori and posterior ethoidectomy and sphenoidotomy done .No active csf leak noted
Meningocele cauterized.
Cribriform defect repaired with layers of fat, ,Gel foam and surgicel reinforced with tissue seal(Tissel).
Left sided nasal packing done with one 8 cm merocele pack.

Right side

No obvious defect visualised

Thinning of bone in the cribriform plate region, 1cm posterior to the axilla.

Fine pulsations noted

This region was cauterised and repaired with layers fat, ,Gel foam and surgicel reinforced with tissue seal(Tissel).

Nasal packing done with one 8 cm merocele pack.

TP shunt insertion done

Treatment Given

Minu Rani was admitted one day prior to surgery after she tested negative for COVID. Her intraoperative period was uneventful.
Pst operatively in the ward she developed breathing difficulty , abdominal pain and chest discomfort and was found to have bialteral wheeze. ECG and cardiac enzymes were normal. Medicine consult was sought. She was suspected to have aspiration pneumonitis and was started on meningitis dose of ceftriaxone, regular nebulisation and 2 litre of oxygen in view of low saturation . Next day she was comforable, hence oxymen was tapered and stopped. She did not have any further episode of desaturation or discomfort. Nasal pack removal was done on post op day 5. She was made to ambulate. She was afebrile for 48 hours, vitals stable and no new complaints, hence she is being discharged with the following recommendations

Recommendations

Tab. Augmentin 1gm BD for 1 week

Tab Pantop 40mg once daily for 1week.

Tab Paracetamol 1g when required with a maximum of three tablets per day.

Tab Levocetirizine 5mg once at night for 2 weeks.

Naturalax powder 2 tbsp with a glass of water as required.

To continue:

Tab. LosartanH (50/12.5) once daily

Tab. Thyroxine 50 mcg once daily

Tab. Metformin+Gp (500mg+1mg)once daily

Tab. Nifedipine R 10 mg once daily at night

To review in ENT 3 OPD on 24/9/2021 for thigh suture removal and 29/9/2021

Please go to Room no. 122 in first floor, Main OPD block, at 11.00 a.m. on 23/9/2021 for abdominal suture removal under Neurosurgery 2 Unit.

No forceful blowing of nose or vigorous rubbing or self-cleaning, starining, coughing or bending and lifting weights.

In case of severe bleeding from the nose/ visual disturbance / swelling around the eye/fever, please come to Emergency Department with this discharge summary. ENT Unit 3 first call will be contacted by the CMO to attend to the call.

THIS SUMMARY HAS NOT BEEN FINALISED. PLEASE ENSURE THAT YOU COLLECT YOUR FINAL REPORT

Written By : Dr. Joyna Singh
Checked By : Dr. Joyna Singh
Verified By : Dr. SHWETHA A


Dr. Lisa Mary Cherian (Associate Professor)
MS., DLO., PhD.,

DEPARTMENT OF ENT UNIT 3
RHINOLOGY AND ANTERIOR SKULL BASE UNIT



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Accredited by the NABH

CHRISTIAN MEDICAL COLLEGE Vellore - 4



APPOINTMENT SLIP



Hospital Number: 223074P Age : 47 Sex : F Name : MINU RANI SHARMA MRS
Appointment Date: 29/09/2021 WED Report to MRO at: 9:30 AM
Service : PRIVATE CONSULTATION REPEAT OPD (WITHIN IN 3 MONTHS)
Clinic : NOSE AND SINUS CLINIC Token No. _____
Location : OPD BUILDING THIRD FLOOR 330
Consultant : LISA MARY CHERIAN Room No. _____
CREDIT SERVICE
Total : Rs.320/- Concess : Rs.0/- Paid : Rs.320/-
Instruction : Invoice No : L6259935
Payment Type : By Credit Receipt No : 54219R169163
Bill No : B066500079 Bill Date : 21/09/2021 11:38 MCTT EmpNo : 54219
Service Code : 34103 Print Date : 21/09/2021 11:38 TransID : 210921034425

Please report to MEDICAL RECORDS OFFICER prior to your Appointment Time.

MAT CODE - 99459